



Specificity Theory: The Evolution of a Process Theory of Psychoanalytic Treatment

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Specificity Theory: The Evolution of a Process Theory of Psychoanalytic Treatment

The author summarizes the perspectives of specificity theory and personal and professional experiences that led to its development. Specificity theory is a contemporary process theory of therapy that holds that each analyst-patient dyad constitutes a unique, reciprocal system, challenging us to reconsider how psychoanalytic therapy is optimally practiced and taught. Therapeutic possibility is co-created in the specificity of fit between the patient's particular therapeutic needs and that therapist's capacity to respond to them, both of which will emerge and change within the unique process of each particular dyad. Specificity theory recognizes that what each therapist effectively offers a particular patient may include but will also transcend considerations of formal theory and its prescribed technique. Evolving against the backdrop of the innovative thinking of Sándor Ferenczi and from the author's personal work with Michael Balint, Wilfred Bion, Donald Winnicott, Marion Milner, and Heinz Kohut, its perspectives are corroborated within contemporary neurobiology, preeminently in the work of Gerald Edelman, and by infant researchers, such as Louis Sander, Ed Tronick, and Karlen Lyons-Ruth.

Every psychoanalytic theory variously reflects significant aspects of its author's personal and professional history (see Stolorow and Atwood 1979). How much more so, then, must a theory of therapeutic specificity do so! I was therefore glad to accept Peter Rudnytsky's invitation to accent these aspects of the theory in presenting my book, *The Power of Specificity in Psychotherapy: When Therapy Works—And When It Doesn't* (Bacal

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2011), at a recent meeting of the American Psychoanalytic Association. My intent, here, is to summarize both the main aspects of specificity theory and the influences on its development.¹

The cover art for the book abstractly depicts two of the major perspectives of specificity theory: (1) that *therapeutic illumination emanates from the emergent process of the particular dyad*, and (2) that *it shines most brightly in a darkened place*. Specificity theory does not provide the psychoanalytic practitioner with do's and don'ts. Rather, it considers therapeutic effect to be centrally a function of the *unpredictable uniqueness of process*, and of the *specificity of dyadic fit*.

Wilfred Bion alluded to the intrinsic uniqueness of unfolding process in psychoanalytic treatment when he wrote, "The only point of importance in any session is the unknown . . . in any session evolution takes place. Out of the darkness and formlessness something evolves" (1967, 273). For Bion, therapeutic effect derives "from the emotional experience with a unique individual and not from generalized theories imperfectly 'remembered.'" Sigmund Freud was, in effect, *discovering* the uniqueness of therapeutic process as he was encouraging his patients to talk to him in ways that neither had ever heard before. However, Freud proceeded to develop a nomothetic, or legislative, theory of human development and of psychological illness, health and cure. That is, Freud's theory involved the formulation of general or universal laws that were premised upon an ordered and structured view of mental life, upon which definitive techniques of psychoanalytic treatment were based. Analysts since Freud have attended to a wider variety of data than Freud did, and in differing ways, but their theories, with few exceptions, still dictated universally prescriptive and proscriptive therapeutic action.

The first chapter of the book addresses the need for a new *kind* of theory. Our familiar *structured* psychoanalytic theories have yielded manifold benefits, and continue to do so. But we need a theory of therapy that accounts not only for the organized, stable, predictable, and universal patterns of mental life, and for its unpredictable, nonlinear patterning; we need a theory that also *gives substance to the unique and specific development and functioning of the mind in process with other minds*. As

Gerald Edelman (2004) has shown, the human mind is continuously formed and transformed through ongoing selective, specific interactions with itself and its environment. Edelman has also demonstrated that when the brain is confronted with novel stimuli, the neuronal connections are actively at work to organize and make sense of the stimuli until a familiar pattern is forged or attained. Our brain is not trying to form a scientifically accurate picture of the world, *per se*, but one that is familiar and meaningful to the perceiver. With this innate biological pull toward closure, curiosity and attention to the novel and unique are blunted. We need a theory of therapy that promotes the efforts of our brain to forestall closure prematurely. Specificity theory helps us counter this inherent tendency—one that *can* be reinforced by the application of structured concepts—and to reengage our curiosity to search out other elements that *may* transcend the relevance of these theories for the patient.

As variously analysand, analyst, and supervisor, I have always been especially interested in what determines, or detracts from, therapeutic effect. In earlier papers (1985; 1998a), I drew attention to what seemed to me to be both obvious and curiously unrecognized, at least officially: that there are multiple ways in which therapeutic effect can be mediated, in addition to interpretive explication, and that this diversity is matched only by the particularity of the patient's therapeutic experience. And then I began to realize that within the clinical approach that these observations informed, there lurked a *theory* that was just as obvious, but which was just as unrecognized: that *effective treatment is a function of the unique and specific, reciprocal process between its particular participants.*²

My colleague, Lucyann Carlton, who has contributed significantly to the development of specificity theory, and to the book, has pointed out that all theories of psychotherapy are premised upon a theory of mind—its development, health, illness and cure; and that while the various schools of psychotherapy offer numerous and contrasting *views* of mind, a fundamental distinction among theories can be drawn from their definitional essence of the nature of the mind either as *structure* or as *process*. It is important to delineate this difference in order to convey specificity theory adequately.

Structure Theory and Process Theory

When the mind is understood as *structure*, it is objectively knowable and develops and functions in predictable ways primarily according to inherent design. When the mind is understood as *process*, it is regarded as *always* in *process*, unknowable in advance. We are, in effect, talking about different *orders* of theory.

Structure theories define the mind in ways that invoke similarities to building or fixing an object such as a steam engine, computer, or multi-story building, with a blueprint for the desired product, and with agreed-upon techniques for its successful completion.

Defining the nature of the mind as structure or process has important implications for how our theories affect our practice. Theories that define the mind primarily as structure offer universally applicable concepts for understanding the patient, along with prescriptive, and proscriptive, guidelines based upon them for responding therapeutically. Structure theories are the type of theory with which we are all familiar as psychoanalysts, theories such as classical psychoanalytic theory, ego psychology, object relations theory, self psychology, and so on.

A theory that defines the mind as process does not offer guidelines for technique or textbooks for understanding a patient. Specificity theory is a process theory that privileges interaction between minds as unique, unpredictable process, yet it retains the potential utility of the multiplicity of our structured perspectives. Established psychoanalytic theories, their concepts, and the approaches they prescribe are regarded as generalities that may or may not illuminate therapeutic process for that dyad. If structure theories are understood within the overarching perspective of the specificity of process, a syncretic use of theory emerges. Structure theories are not discarded by specificity theory. Structure theories are useful in at least two ways: they provide a particular structure to consider, and they may serve as a probe to expand our observational field. At the same time, they may occlude aspects of the observational field and impose a structure that is not applicable specifically to a particular patient/therapist pair.

Therapeutic action, thus informed, may be imbued with one or another structured concept, or “model,” but these models will come into play in a different way. They will emerge and be understood within the overarching perspective of the *specificity of process of that dyad*. This can help the therapist to keep his or her mind open, both figuratively and literally, to what may usefully emerge in process, to counter the mind’s tendency to closure.

Such process centrally entails *mutual selectivity*. This is another way of describing what we have loosely called “fit,” which we intuitively, though informally, recognize as crucial for therapeutic effect. This entails, as Louis Sander has described, “a sort of resonance between two systems attuned to each other by corresponding properties,” or matching specificities (2002, 19).³

The primary consequence of adopting a process view of mind rather than a structure view of mind is a *radical shift in epistemology*. This is a formal way of saying that a process view of mind changes both *how* and *what* we, as psychoanalysts, know in working with a patient. When specificity becomes central to our consideration of process, the epistemological shift preeminently illuminates *how the who* of the participants in the particularity of their process will influence therapeutic effect. From the *process* perspective of specificity theory, therapeutic effect is co-created—or not—within the unique, unfolding process of each *particular* psychoanalytic dyad. Put in yet another way, it is ultimately the capacities and limitations of *that* therapist and *that* patient to understand and respond to each other in their process that determines therapeutic effectiveness. Our book includes illustrations of how specificity theory is applied in clinical practice and in the supervisory setting. Examples also illustrate how specificity theory alters our view of psychoanalytic concepts and principles and how this affects therapeutic action.

Partly because of a mischievous streak in my personality, I will briefly describe the psychoanalytic treatment of a young man who was pretty well cured in little over three months by my helping him get out of analysis. This case vignette is actually not in the book. The treatment happened a long while ago, yet as I look back on it, it seems to me to be a nice illustration of certain aspects of specificity theory.

Eric was around twenty years old when he was referred to me, about forty years ago, by a prominent psychoanalytic colleague. Eric's father, an attorney in the city, had asked my colleague to find Eric a good analyst. He was very concerned about his son's state of mind and the direction his son's life was taking. Eric was hardly studying at all for the courses that would enable his admission to law school, and he seemed constantly irritated and depressed. Instead of applying himself to his schoolwork, he spent all his time dabbling in "ridiculous" business ventures. Eric's father not only knew that Eric could benefit from analysis, he also knew what sort of analysis would best benefit Eric. He himself had completed a ten-year analysis some years ago, in London, with Hanna Segal, a senior Kleinian analyst. He had found this to be enormously valuable, and he knew his son would likewise benefit from the same sort of treatment.

From the referring analyst's point of view, I may have looked like the nearest thing in sight to a Kleinian analyst in San Francisco at that time. I had qualified about six years previously from the Institute of Psychoanalysis in London, where two of my six supervisors were senior Kleinian analysts—Bion for an adult case and Betty Joseph for a child analytic case—and where Hanna Segal, herself, was one of my instructors in Kleinian theory. So, I knew "Klein," but I was not a "Kleinian." I'd been affiliated with the so-called Middle Group of the British Society, which was a peculiar non-group group. The Middle Group, which sometimes called itself the Independent Group, was supposedly poised somewhere between Freudian and Kleinian sensibilities, but mainly comprised a coterie of creative, independent thinkers, several of whom I had trained with, such as Michael Balint, Donald Winnicott, Marion Milner, John D. Sutherland, and Charles Rycroft.

I really liked Eric—a straightforward, congenial, bright, and ambitious young man. Eric and I connected well, and we settled in to a four-times-per-week analysis. I recognized, though, not long after our first few sessions, that the best kind of analysis we could do was to engage the issues that could get him out of analysis as soon as possible and allow him to get on with his life.

Psychoanalytic theory that emerged for me during our process turned out not to be Kleinian (though Klein's concepts did variously inform my work then, and continue to do so now). What emerged as particularly useful in my work with Eric was a concept that had been advanced by Harry Guntrip in the late sixties. What Bernard Brandchaft (2004; 2007) later came to elaborate as "pathological accommodation," Guntrip (1968) had first conceptualized in object relational terms as a dilemma centering around the experience of a conflict wherein the needs of the self were threateningly at variance with those of a significant figure in the person's life. To express one's self was to lose the object. To comply with the other was to lose one's self. Eric was struggling with his father's conviction that becoming a lawyer would unquestionably be right for him. Eric and I came to appreciate that were he to accede to his father's wishes, he faced the dangers of losing his sense of purpose and the invalidation of his core ambitions; but that if he did not accommodate his father's wishes, he would, at the very least, seriously displease him. His depressiveness and irritability reflected this unconscious conflict symptomatically.

My not-so-unconscious attitude, which would seem to have specifically connected with Eric's, as our process unfolded, was that pursuing what was centrally meaningful in Eric's life—going into some kind of business, rather than studying to become an attorney as his father required of him—trumped Eric's dread of losing his father's approval and respect. I must have talked with Eric in ways that implicitly gave expression to this understanding. My interpretations about Eric's conflict around this may well have been therapeutically significant; yet what was, arguably, at least as effective was how Eric and I found ourselves able to *be together*, in effect, to fit together, therapeutically, *around this issue*.

What was emerging, within me, in our process, were precipitates of toxic experiences with my mother that were specifically analogous to those with which Eric struggled in relation to his father. These experiences with my mother had been repeated in my recent analysis, with Michael Balint. In that analysis, I felt little recognition of who I was other than being identified as faulty for not measuring up to his standards

of mental health. Because Eric wanted to be who *he* could be rather than what his father regarded as right, he was regarded as unhealthy, and therefore should have an analysis.

I now look back on my objections to these attitudes on the part of my first official analyst, and before that, on the part of my first unofficial one, my mother, as the anlage of specificity theory, in the sense that at some level I recognized that neither dyad was a very good *fit*. Until a later analysis, however, I could not experience my considerable protests against any of this as valid for myself.

Eric was luckier than I had been. Unconsciously, or perhaps dissociatively, I must in some measure have registered the validity of my protests, since I could access this sensibility, with feeling—along with a different attitude, and a different sort of responsiveness to Eric—in the specificity of *our* process. In some measure, Eric and I unconsciously connected to stand firm against an unempathic, omniscient, and tyrannical parent.

Traditionally, this would be conceptualized as a transference/countertransference enactment. I, however, locate my connection with Eric under the rubric of the specificity of dyadic process, which designates something more than what is encompassed by the traditional concept—that is, the scope of therapeutic possibility for a particular dyad. A significant aspect of Eric's therapeutic experience with me, I believe, was that I could listen, with sincere interest, to his "ridiculous" ideas for developing commercially viable projects. I was, in retrospect, also finding myself responding to Eric in a way that Kohut would later conceptualize as that of a mirroring selfobject. However, I knew nothing formally, then (in 1971), about self psychology theory. I actually did not come across Kohut's theories for another five years.

Back, now, to Eric and me. After about three and a half months, Eric and I decided we'd gotten to the point where he could get off the couch and head for the door. As he was leaving my office, he turned toward me, and said, with a wide smile on his face, "It was good doing business with you, Doc."

Returning now, to my own analysis, with Balint. Not only did his proclamations about me not feel good, what he was doing didn't feel quite right. In one session, I found myself imploring him to relate to me in a different way. Rather than

hurl interpretations at me from up there, could we be like two children sitting on the floor side by side, leaning against the couch looking at something together and discussing what each of us saw in it? My intent with this metaphor was to elicit the sort of responsiveness that I felt would allow me to have a therapeutic experience—comprising not only a nonjudgmental attitude but also a sharing of what might interest us together. I have come to learn that this is particularly important for me. My suggestion to Balint that there might be a better way that this activity called analysis could be carried out with *me* was, in retrospect, also my first implicit attempt to field a theory of therapeutic specificity.

I had a very different, very useful, experience with Balint's supervision of my first "control" case. This supervision was conducted in accordance with the Hungarian method of psychoanalytic supervision, which takes place in the setting of the candidate's analysis. I will return to this shortly.

There was no one I felt I could talk to about my dysphoric analytic experiences. In some desperation, I drafted a "paper," toward the end of that first year of analytic training, which I titled "Is Analyzing Necessary?"—my second incipiently creative attempt to draw attention to *different* ways of experiencing therapeutic effect. I assessed, probably wisely, though, that if I were to bring this little treatise to the light of day, I might not get much further in my analytic training school! I have since wondered whether such specificity may have been a significant factor in Harry Guntrip's experience in analysis with Fairbairn, and then with Winnicott (Guntrip 1975). Analogous to Guntrip's later more helpful analytic experience with Winnicott, I found a better fit with Heinz Kohut (also see Bacal 2006), which I will come to in a moment.

My interest in the specificity of therapeutic process was significantly evoked by the methodology of a psychotherapy outcome research project along with one of its unanticipated findings. This project, in which I participated during the same time period that I was undertaking analytic training in London (in the '60s), was conducted under the leadership of David Malan (see Malan, Bacal, Heath, and Balfour 1968) at the Tavistock Clinic. It entailed the assessment of the psychological status of a series of untreated "neurotic" patients who had

been seen at intake at the Clinic at least two years previously. Our aim was to explore at a follow-up interview the extent to which improvement had occurred without formal treatment.⁴ We considered the attainment of true change neither on the basis of relief of symptoms nor on the basis of the usual general categories of healthy social and psychological functioning, but rather on significant amelioration in specific aspects of the patient's life situation that appeared to reflect change in his or her susceptibility to experiences that stressed particular vulnerabilities.

While we often did not agree when it came to formulating the patient's dynamics on the basis of traditional psychoanalytic concepts (even though our theoretical differences were minimal), we had little trouble reaching a consensus on what would constitute improvement for the particular patient. Our study (Malan, Heath, Bacal, Balfour 1975) also indicated the operation of an unpredictable process of specificity. It turned out that those who had improved most on psychodynamic criteria had either experienced significant therapeutic effect from particular experience in their initial consultation⁵ or/and were subsequently able to use similar, specific responsiveness from others, therapeutically. While this research illuminated the specificity of outcome for the particular individual, it also suggested that therapeutic process that led to whatever outcome is less dependent upon application of any particular formal theory than it is a function of the specificity of dyadic interaction.

Experience with Supervisors

Experience with certain analytic supervisors also stimulated my thinking about the power of specificity in psychoanalytic practice and supervision. These preeminently included Michael Balint, Wilfred Bion, Donald Winnicott, and Marion Milner.

Michael Balint

Balint supervised my first analytic control case for seven and a half years according to the Hungarian psychoanalytic

training system, wherein the training analyst supervised the candidate's first case within the candidate's own analysis, as he shares his experience of his patient in the course of his associations. I found this form of supervision to be immensely valuable, as it enabled me to appreciate, with striking immediacy, how specific aspects of who I was, in context, affected my responses to my patient. I found this experience to be remarkably different from those I had when Balint directed his understanding to me alone, in the analysis. A curious situation, the determinant for which I suggested was a significant lack of fit between how he was responding to me and what I felt I needed, therapeutically. In the context of the supervision, though, there *was* a fit. We *shared* something that really interested both of us. I have also wondered whether another determinant for the difference in what unfolded in these two settings could have been a psychoanalytic version of the familiar phenomenon of the heightened tolerance of the grandparent for his grandchildren over that for his own children. In this context, the grandparent was Balint to his trainee-analysand's patients—that is, to my analysands. My analytic grandfather, in this context, was Sándor Ferenczi, with whom, I suspect, I might have experienced a more therapeutic fit.

Balint Groups

In a related setting, I learned from Balint about the importance of specificity of interaction for pedagogic as well as for therapeutic effect through conducting discussion groups for general practitioners in the psychological aspects of their work. These groups, which Balint originated and were further developed by Michael and his wife Enid, also a psychoanalyst, strikingly illuminated the remarkably variegated ways that patients experience their doctors as therapeutic, and how particular doctors could or could not work effectively with certain patients (M. Balint 1957; 1969; M. and E. Balint 1961). These groups, which came to be called “Balint Groups,” are a form of applied psychoanalysis (E. Balint 1967). Their central task comprises the study of the therapeutic effect of the *particular* patient-doctor relationship. As Enid Balint observed, on reviewing Michael Balint's work with a research group of general practitioners:

[Our aim was] to have a wider look at the . . . uncertainties which abound in working with patients; while at the same time not attempting to reduce them to neat patterns, leaving the perspectives as wide as possible in the realization that what one person sees from one angle cannot be seen from another's . . . the doctors [came to] realize that their healing powers were strictly limited, but . . . on the other hand, observed that very small changes in a person can produce amazingly large effects on the life of an individual. It seems that the unique relationship which occurs in the general practice setting informs us as to what the work is really about. (E. Balint et al. 1993, 98–99)

Wilfred Bion

During a substantial portion of my supervision with Wilfred Bion, he was focusing on perspectives that he was writing about in his paper, “Notes On Memory and Desire” (1967). In this paper, Bion recommended that the analyst expunge any prior knowledge or conception of the patient he is about to see, except his name and the hour that he is expected to appear for his appointment. Bion famously remarked that if you feel you’ve seen the patient before, you’re treating the wrong patient. I could never quite emulate this approach, but it impressed upon me the value of sustaining a truly not-knowing state of mind, and drew me more deeply into attending to the uniqueness of the patient as this impacted me as the analyst.

Donald Winnicott

Donald Winnicott, as many of us recognize, espoused a clinical approach that richly foreshadowed that of contemporary relational self psychology (Bacal 1989; Bacal and Newman 1990). Like Ferenczi, Winnicott also recognized—and conveyed to me—that what may be most important to the patient is not what the analyst interprets but how the analyst behaves. I remember Winnicott telling me, some time in 1967, how impressed he was with a young social worker whom he observed treating a child when on a recent visit to New York. Had I heard

of her, he asked? Her name, he told me, was Virginia Axline, and she had written a remarkable book, *Dibs in Search of Self* (1964), in which she effectively treated a child “without making any interpretations at all!” (Actually, it seems that she did, but this is not what impressed Winnicott about how Axline was therapeutic with her young patient.) Winnicott told me a similar story about Melitta Schmideberg, the physician-psychanalyst daughter of Melanie Klein, who had taken a job working with psychotic children at a mental hospital north of London shortly after the Klein family emigrated to England in the late ‘20s. “She knew not a stitch of English at the time,” Winnicott told me, “yet her therapeutic effect on those children was amazing” (see Bacal 2011, 47).

I vividly recall the preliminary meeting I had with Winnicott when I was about to begin the analysis of a three-and-a-half-year-old girl under his supervision. The child’s behavior had become chaotic and bizarre in the situation of the meager one-room flat in which she lived with her father, her mother, and her two-year-old sister. I asked Winnicott how to begin the treatment (as I had never worked with a child before and had not a clue what to do, especially as the child was refusing to leave the waiting room to accompany me to the consulting room). Without hesitation, Winnicott answered, “If she holds out her hand, take it.” I have never forgotten this. It became a metaphor not only for the responsivity that enabled me to be therapeutic for my little patient but for my work with adult patients as well.

Marion Milner

I had similar experience of Marion Milner’s daring to think quite differently about what a *particular* patient might need. During her supervision of one of my training cases, she understood my sensitive patient’s retreat into the woods as a young adolescent, where he would lie for hours in holes he would dig in the ground, as a way to find himself and to grow, rather than as some kind of schizophrenic or seriously schizoid withdrawal, as previous psychoanalysts and psychiatrists had diagnosed. Milner suggested that I might apprehend his behavior as tantamount to the beginning growth of a delicate

young plant that needs careful tending as it begins to sprout new shoots.

This sort of supervision of my patients was, understandably, also stirring a good deal of conflict in me about how analysis does cure, as my own analytic experience, which was going on at the same time, was quite different. It was to take me about twenty-five more years before I truly appreciated that being confronted with my evident defects was not going to be optimal for *my* personal development. It was only then that I could begin seriously writing about how I thought psychoanalytic treatment could be therapeutic (Bacal 1985).

How Does Analysis Cure? Beyond Kohut's Two-Step Paradigm

My second analytic experience, with Heinz Kohut, some years later,⁶ went much better than the first one, although I came to differ somewhat with Kohut, in both theory and practice, about "how analysis cures" (see Kohut 1984). In the early days of my analysis with Kohut, I discovered, as a kind of revelation, really, the therapeutic power, for me, of empathy as *response*, which Kohut, in his writings, actually disavowed, asserting that empathy constituted only a data-gathering instrument. Yet, that was how I experienced Kohut's fairly consistent responsiveness to me. I was not *used* to this, and it took me some time to get over experiencing this as somewhat indulgent.

Kohut described two distinct yet overlapping steps in therapeutic action, which comprised a methodology that was, in effect, quite distinct from his self theory. These two steps were: empathic immersion in the patient's subjective experience, followed by explanation, that is, interpretation, of what the analyst had gleaned thereby. My sense was that Kohut needed to believe—as the "classical" analyst that always, in part, constituted his ego ideal—that the second step, interpretation, was essential to therapeutic cure. Kohut's responsiveness to me, and also something he said, once, indicated to me that he did not actually subscribe to this. Kohut told me that he thought "Ferenczi had the right idea." That meant, quite clearly, in the context of the conversation we were having, that Kohut,

like Ferenczi, believed that there was a good deal to be said for responding to a patient in ways the *patient* experienced as therapeutic, even if these ways would not be endorsed by traditional psychoanalysis—and, officially, not by him either. Kohut did offer me many carefully considered interpretations, but what positively affected me was not this understanding, as accurate as it could be—actually his interpretations were much too long—what has stayed with me was the accepting quality of his presence, along with kindness and optimism, which accorded with what *I* needed at the time. I also experienced Kohut's explicit responses, *other* than interpretation and empathy, as therapeutic; one of the most salutary of these was an encouraging arm around the shoulder at an especially dispiriting time when we were facing yet another two-week break.⁷

I am not the only one who benefited from Kohut's inconsistency in adhering to his declared two-step procedure for therapeutic action. There are two published examples of this that are quite striking. Perhaps the most famous one is that of Kohut's patient, Miss F—a case that is often cited as template for the discovery of self psychology (see Kohut 1971, 283–90). Miss F was a patient for whom the echoing of her subjectivity was the only response she experienced as therapeutic; anything else provoked catastrophically self-disorganizing rage. Kohut could not help her for some time, as he was wedded to a way of working that prioritized conceptualizing and delivering verbal understanding, that is, interpreting. It was to Kohut's credit that he was ultimately able to shift from this mode to one of simply providing a mirror for her experience, which turned out to be remarkably effective. We note that Kohut's response to Miss F that was useful to her was neither that of interpretation nor empathy.

The other—almost equally well known—example is that of Kohut's offering a patient two fingers to hold at the point when he feared she was becoming seriously suicidal (Kohut 1981). Kohut tells us that as he did this and the patient took hold of his fingers, he made an interpretation—to himself—“because an analyst always remains an analyst. . . . It was the toothless gums of a very young child clamping down on an empty nipple” (535). In fact, Kohut did not, in this situation, make an interpretation to the patient, nor was he offering his

patient empathy as therapeutic response. But because analysts define themselves as analysts by virtue of their adherence to the belief, or doctrine, that only interpretation can “cure,” an interpretation had to figure somewhere in Kohut’s response, even if articulated silently. In effect, the action he deemed therapeutic was that of offering his two fingers for his patient to hold onto. Kohut understood via his empathic attunement to this patient that, for her, at this time, her *experience* of being apprehended (her experiencing his empathic attunement to her lifelessness) could not be mediated through his verbal expression of his feeling for her. The patient needed to experience Kohut’s extension of himself and then the opportunity to hold on. Offering the two fingers was not an interpretation, but it did, in our view (see Bacal and Carlton 2010), constitute the practice of psychoanalysis at its best, exemplifying Kohut’s ability to understand and move to the patient’s therapeutic need.⁸ Our view is that it was also self psychology, specifically utilized, at its best. While Kohut’s empathic understanding and associated response were informed by the structure of his theory, they were not limited by the constraints that would direct him to interpret the patient’s archaic selfobject need rather than to provide it, symbolically, in this particular instance.

I explicitly gave substance to a different version of Kohut’s two-step paradigm for therapeutic action—empathic understanding, followed by interpretive explication—when I altered the second step to *optimal responsiveness* (1985; 1998a). Interestingly, the more my work was guided by the expansion of Kohut’s second step, explanation, in therapeutic cure to optimal responsiveness, the more I felt challenged to explore the first step, empathic understanding, because this step, in effect, constitutes a process of discovery of what the particularity of response might be that the patient may experience as therapeutic. That is, in recasting Kohut’s second step in this way I was, in effect, deepening my understanding of Kohut’s first step (empathic attunement) with the recognition of the inherent specificity of the empathic method. I did not alter this first step as articulated: empathically informed understanding. But, with Carlton, I began, through specificity theory, to challenge what is meant by the *process* of understanding (Bacal and Carlton 2008; 2010).

With the development of a theory of the specificity of process, that is, that *therapeutic effect is specific to the particular analyst-patient pair as it might emerge—or not—in their process*, we now propose that the two steps, understanding and response, not only overlap; they are inextricably linked, one with the other. We must now acknowledge that the measure of psychoanalysis as a therapy is neither “empathy” nor “response,” but rather the fullness and complexity of both steps. In other words, the *sine qua non* of psychoanalytic therapy is not empathy culminating in verbal interpretation as the response in the second step, but the entire process that includes understanding the complex mental states of another and discerning responses that may be *therapeutic for that patient*, and *possible for that analyst*, in the uniqueness of their unfolding process.

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Notes

1. Chapter 5 in the book describes more fully the influences on my development of specificity theory.
2. My focus is on how specificity theory applies to the dyadic situation of psychoanalytic treatment, and to psychoanalytic training; but its perspectives are also applicable to other settings, such as couples therapy, group psychotherapy, and family therapy.
3. The biologist, Paul Weiss, as quoted by Sander (2002, 19), describes the essential principle in establishing and maintaining the connections upon which the coherence of living organization depends as one of “matched specificities—a sort of resonance between two systems attuned to each other by corresponding properties” (162).
4. Our intent was to establish a baseline in order to challenge H. J. Eysenck’s (1952) hypothesis that, in psychotherapy, one-third of patients will improve, one-third will get worse, and one-third will remain the same. On the basis of our study of “untreated” patients, Eysenck turned out to be right, on symptomatic criteria, but not on psychodynamic criteria (Malan, Bacal, Heath, and Balfour 1968).
5. Interestingly, Michael Balint was the consultant who did the intake interview with a patient whom our research team considered to be one of the most severely and chronically disturbed individuals in our cohort. Balint confronted this patient with the way she was wasting her life. He told her that she needed to take definitive steps to change her ways if she wanted a chance to live differently. This patient went on to “use” a particular psychologist relative similarly, though informally. He confronted her in the same way Balint had done; she turned out to have the most successful “outcome” in our series of formally “untreated” patients.
6. I saw Balint between 1961 and 1970, and Kohut between 1976 and 1981.
7. I was seeing Kohut for three sessions every other week, traveling to Chicago from Toronto.

8. How this patient actually experienced what Kohut did we cannot know; and analysts have, since that famous moment, debated whether he could have done just as well by interpreting her need for human contact; or, some have felt, perhaps she would have done even better had Kohut offered her his whole hand rather than just a couple of fingers.

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